LOCALOBS GYNAE (Part 1)

RECALL JULY 2026

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Anatomy

1. Diagonal conjugate is measured from sacral promontory till?

A. Postero-superior border of symphysis pubis

B. Postero-inferior border of symphysis pubis

C. Inferior border of symphysis pubis

D. Pubic arch

2. Mons pubis lies on?

A. Pubic symphysis and surrounding pubic arch

B. Urogenital diaphragm

C. Perineal membrane

D. Deep perineal pouch

3. Muscle that forms the major component of levator ani is?

A. Iliococcygeus

B. Pubococcygeus

C. Coccygeus

D. Pubovaginalis

4. External urethral sphincter in female lies on?

A. Superficial perineal pouch

B. Deep perineal pouch (urogenital diaphragm)

C. Levator ani muscle

D. Douglas pouch

5. Lymph drainage of the cervix primarily drains to?

A. Superficial inguinal LN

B. Deep inguinal LN

C. Para-aortic LN

D. Internal and external iliac LN

6. Lymph drainage of the body of uterus primarily goes to?

A. Para-aortic LN

B. Superficial inguinal LN

C. External inguinal LN

D. Deep inguinal LN

7. Sensory sensation of the anterior part of the labia majora is from?

A. Ilioinguinal nerve

B. Pudendal nerve

C. Genitofemoral nerve

D. Posterior cutaneous nerve of the thigh

8. Dorsal nerve of clitoris is a branch from?

A. Pudendal nerve

B. Ilioinguinal nerve

C. Obturator nerve

D. Femoral nerve

9. The fundus of uterus during dysmenorrhea pain goes to sympathetic through?

A. Pelvic splanchnic plexus

B. Inferior hypogastric nerve

C. Superior hypogastric plexus

D. Lumbar splanchnic nerves

10. Hymen is the remnant of which structure?

A. Mesonephric duct

B. Paramesonephric duct

C. Urogenital sinus

D. Genital tubercle

11. The main support of uterus is by?

A. Ovarian ligament

B. Cardinal ligament

C. Suspensory ligament of ovary

D. Broad ligament

12. Cardinal ligament is transverse cervical ligament which attaches from cervix to?

A. Broad ligament

B. Lateral pelvic wall

C. Sacrum

D. Ischial spine

13. Suspensory ligament of ovary contains which structure?

A. Ovarian vessels, lymphatic and autonomic fibers

B. Ovarian vessels

C. Uterine artery

D. Round ligament of uterus

14. Inferior rectal artery which innervates external anal sphincter is a branch from?

A. Internal iliac A

B. Pudendal A

C. Femoral A

D. Inferior mesenteric A

15. Structure that passes through greater sciatic notch below piriformis muscle?

A. Superior gluteal Artery

B. Femoral Nerve

C. Pudendal Nerve

D. Obturator Artery

16. The puborectalis sling, part of levator ani muscle that supports anorectal canal, defect in which part can cause fecal incontinence?

A. Iliococcygeus muscle

B. Pubococcygeus muscle

C. External anal sphincter

D. Puborectalis

17. Ischiocavernous muscle in female attaches to which structure?

A. Clitoral body

B. Crura of clitoris

C. Vaginal wall

D. Perineal body

18. Which gland in female is homologous to Cowper gland (bulbourethral gland) in male?

A. Greater vestibular gland (Bartholin)

B. Paraurethral gland

C. Prostate

D. Crus of clitoris

19. Anesthesiologist wants to do pudendal nerve block around ischial spine why?

A. Pudendal nerve runs superior to ischial spine

B. Pudendal nerve runs in pudendal (Alcock's) canal in lateral wall of ischioanal fossa

C. The ischial spine is an easily palpable bony landmark near the sacrospinous ligament

D. The pudendal nerve crosses the posterior surface of the ischial spine as it leaves the greater sciatic foramen

20. Pelvic inlet is part where entering pelvis, it is bounded anteriorly by?

A. Sacral promontory

B. Upper border of symphysis pubis

C. Inferior border of symphysis pubis

D. Arcuate line

Physiology

21. The Pathophysiology of type 2 diabetes is:

A. Autoimmune destruction of insulin receptors

B. Peripheral insulin resistance

C. Autoimmune destruction of pancreatic beta cells

D. Absolute insulin deficiency from birth

22. Patient has recurrent hx of DVT, investigation shows that he has protein C deficiency. What is the function of protein C?

A. Inhibition of prothrombin

B. Activation of fibrinolysis through thromboplastin

C. Activation of factor X

D. Inactivation of factor Va, VIIIa

23. HCG hormone is similar in structure to?

A. FSH

B. LH

C. GH

D. TSH

24. Patient with chronic renal failure his ABG as follow: Hco3 18, pH 7.2, Co2 24. What is his pattern?

A. Respiratory alkalosis

B. Metabolic acidosis with proportionate respiratory compensation

C. Respiratory acidosis with Metabolic compensation

D. Fully compensated metabolic acidosis

25. Estrogen maintains the pH of vagina in between?

A. 6-7

B. 5-6

C. 4-5

D. 3-4

26. Inhibin B is substance secreted from which cell and during which phase?

A. Granulosa cell during follicular phase

B. Theca cell during luteal phase

C. Granulosa cell during luteal phase

D. Theca cell during follicular phase

27. Plasma volume during pregnancy increases by what percentage?

A. 30%

B. 50%

C. 90%

D. 60%

28. Comparing to non-pregnant women, cardiac output in pregnant women would be increased by a range of?

A. 10-15%

B. 20-25%

C. 30-50%

D. 40-60%

29. What is the first change that is observed in atherosclerosis?

A. Thickening of blood vessel due to smooth muscle migration

B. Fibrous plaque

C. Fatty streak

D. Formation of foam cells

30. By which mechanism there is an increase in body temperature 0.5°C post-ovulation?

A. Progesterone increases the hypothalamic temperature set-point (thermogenic effect)

B. Corpus luteum increases metabolic rate due to high vascularity

C. Estrogen increases temperature for follicular rupture

D. LH surge directly acts on peripheral tissues to produce heat

31. Main Estrogen produced by the ovary is?

A. E1 (Estrone)

B. E2 (Estradiol)

C. E3 (Estriol)

D. E4 (Estetrol)

32. Why is the transformation zone higher in the endocervix in reproductive age?

A. High progesterone levels cause atrophy of the squamous epithelium

B. High estrogen levels stimulate cervical eversion

C. Local pH changes in the vagina

D. Mechanical due to intercourse

33. The anovulatory bleed seen in PCOS differs from true menstruation in?

A. LH surge-induced prostaglandins release

B. Estrogen withdrawal bleeding without prior progesterone priming

C. Progesterone withdrawal after a normal luteal phase

D. Elevated FSH causing corpus luteum regression

34. Anti-Mullerian hormone (AMH) is produced by granulosa cells of:

A. Large antral follicles and the dominant follicle

B. Small antral and preantral follicles (2–8 mm), absent in primordial and very large follicles

C. Atretic follicles undergoing apoptosis

D. Primordial follicles prior to recruitment

Embryology

35. The Mullerian duct (Paramesonephric duct) will give rise to?

A. Uterus, fallopian tube, cervix, upper 1/3 of vagina

B. Ovaries

C. Lower 2/3 of vagina

D. Labia majora and clitoris

36. Germ cells will be arrested in which phase of meiosis I and complete after what?

A. Prophase I, after entry of sperm

B. Metaphase, during ovulation

C. Dictyotene stage of Prophase I, resumed after LH surge

D. Dictyotene stage of Prophase I, completed at menopause

37. About ovarian reserve, what does that mean?

A. Number of mature Graafian follicles in the ovary

B. Total number of primordial follicles found in ovary

C. Ovarian stroma

D. Number of Oogonia at birth

38. In which of these events occurs completion of meiosis II?

A. Sperm head decondensation after penetrating zona pellucida

B. Sperm penetration of oocyte plasma membrane and release of intracellular Ca in ova

C. Binding of the sperm to the ZP3 receptor

D. Selection of the dominant follicle during the luteal phase

Biochemistry

39. Allosteric enzyme of glycolysis is?

A. Hexokinase

B. Phosphofructokinase-1 (PFK-1)

C. Pyruvate kinase

D. Phosphoglucomutase

40. Most common cause of congenital adrenal hyperplasia is?

A. 11-beta hydroxylase deficiency

B. 17-alpha hydroxylase deficiency

C. 21-hydroxylase deficiency

D. 3-beta-hydroxysteroid dehydrogenase deficiency

41. A 10-year-old child presents with DKA. In his urine there is acetoacetate. What is the key enzyme for ketone body synthesis?

A. HMG-CoA lyase

B. HMG-CoA synthase

C. Acetyl-CoA carboxylase

D. Thiolase

42. The urea cycle occurs partly in the mitochondria. Defect or deficiency in which component leads to hyperammonemia with high orotic acid and low citrulline?

A. Carbamoyl phosphate synthetase 1

B. Ornithine transcarbamylase

C. Argininosuccinate synthetase

D. Arginase

43. Which amino acid is the main carrier for nitrogen from peripheral tissue to liver during fasting?

A. Glutamate

B. Aspartate

C. Alanine

D. Glutamine

44. Pentose phosphate pathway is a main source of coenzymes in RBCs. What is the final primary product?

A. NAD+

B. FAD

C. NADPH and ribose-5-phosphate

D. ATP and NADH

45. Female with gouty arthritis is on allopurinol and develops xanthinuria. Allopurinol inhibits which of the following enzymes?

A. Xanthine oxidase

B. Hypoxanthine oxidase

C. Adenosine deaminase

D. PRPP synthetase

46. Pellagra disease is characterized by the 4 D's: Diarrhea, Dementia, Dermatitis, and Death. The cause of this disease is a deficiency of:

A. Vitamin B1 (Thiamine)

B. Vitamin C

C. Vitamin B3 (Niacin)

D. Vitamin D

47. Newborn presents with ambiguous genitalia with a 46,XX genotype. Which enzyme deficiency can be the cause of this condition?

A. 11-beta hydroxylase

B. 21-hydroxylase

C. 17-alpha hydroxylase

D. 11-alpha hydroxylase

48. In fatty acid synthesis, when acetyl-CoA transfers from mitochondria to cytoplasm, it converts to Citrate by:

A. ATP citrate lyase

B. Acetyl-CoA carboxylase

C. Isocitrate dehydrogenase

D. Citrate synthase

49. Neonate with cataracts, jaundice, and hepatomegaly is diagnosed with Galactosemia. Which enzyme deficiency causes this presentation?

A. GALT (Galactose-1-phosphate uridylyltransferase)

B. Galactokinase

C. Hexokinase

D. Aldose reductase

50. Patient with macrocytic anemia develops subacute combined degeneration of the spinal cord. Investigations show normal folate and low vitamin B12. What is the mechanism of neurological symptoms?

A. Homocysteine increase in blood causing endothelial damage

B. Decreased Methionine synthesis impairs myelin sheath maintenance

C. Accumulation of methylmalonic acid (MMA) in the kidneys

D. Direct autoimmune destruction of the dorsal columns

51. A 9-year-old boy is brought to the clinic due to persistent bone pain and easy bruising. Physical examination reveals massive enlargement of the spleen. A bone marrow biopsy reveals lipid-laden macrophages with a cytoplasm that resembles crumpled tissue paper ("Gaucher cells"). Which enzyme is deficient?

A) Sphingomyelinase

B) Glucocerebrosidase

C) alpha-Galactosidase A

D) beta-Galactocerebrosidase

Pathology

52. Patient with non-caseating granuloma with bilateral hilar lymphadenopathy is found to have increased ACE in serum. The most likely diagnosis is?

A. Histoplasmosis

B. Sarcoidosis

C. Tuberculosis

D. Coccidioidomycosis

53. To differentiate between Follicular carcinoma of the thyroid and follicular adenoma on biopsy, you must show:

A. Lymphocytic infiltrate

B. Capsular and/or vascular invasion

C. Psammoma bodies

D. Anaplastic giant cells

54. Post-mortem of a 70-year-old female who died with congestive heart failure shows a "nutmeg liver" on autopsy. The underlying cause is:

A. Centrilobular hemorrhagic necrosis/ischemia

B. Generalized hepatic steatosis

C. Periportal massive fibrosis

D. Primary biliary cholangitis

55. Patient with a renal mass and hematuria. Biopsy shows clear cells with abundant glycogen and lipids that wash out during processing, surrounded by a rich vascular stroma. What is the cellular source of this tumor?

A. Medullary collecting duct

B. Glomerular endothelium

C. Renal proximal convoluted tubule

D. Renal pelvis transitional epithelium

56. An 8-year-old boy presented with periorbital edema and hypoalbuminemia, diagnosed as nephrotic syndrome. Which pathology is most commonly related to this age group?

A. Minimal change disease

B. Focal segmental glomerulosclerosis

C. Membranous nephropathy

D. IgA nephropathy

57. A 40-year-old alcoholic man presents with upper GI bleeding. On endoscopy, there is a longitudinal laceration in the area of the gastroesophageal junction. The most likely cause is:

A. Boerhaave syndrome

B. Mallory-Weiss syndrome

C. Portal hypertension

D. Barrett's esophagus

58. A 30-year-old female complaining of muscle weakness and fatigability is found to have a mediastinal mass (thymoma). What is the likely diagnosis?

A. Lambert-Eaton Myasthenic Syndrome (LEMS)

B. Myasthenia gravis

C. Polymyositis

D. Dermatomyositis

59. Female with rheumatoid arthritis affecting the kidney. Analysis of the amyloid protein under Congo Red stain yields the classic apple-green birefringence under polarized light microscopy. What is the main protein deposit?

A. AL (immunoglobulin light chain)

B. AA (amyloid associated, derived from SAA)

C. Beta-2 microglobulin

D. Transthyretin

60. Lady with a skin lesion on her nose that bleeds easily on touch, has many telangiectasias, and contains basaloid cells on biopsy. The most likely diagnosis is?

A. Squamous cell carcinoma

B. Melanoma

C. Basal cell carcinoma

D. Merkel cell carcinoma

61. A 19-year-old female with a painless neck swelling. After excision, a cystic lesion containing cholesterol crystals is found. What is the most likely diagnosis?

A. Thyroglossal duct cyst

B. Cystic hygroma

C. Branchial cleft cyst

D. Dermoid cyst

62. Smoker with a central lung mass. Biopsy reveals keratin pearls and intercellular bridges. What is the diagnosis?

A. Small cell carcinoma

B. Adenocarcinoma

C. Squamous cell carcinoma

D. Large cell carcinoma

63. Young female presents with progressive weakness and sensory loss. MRI shows multiple white matter lesions separate in time and space demonstrating demyelination. What is the most likely diagnosis?

A. Central pontine myelinolysis

B. Progressive Multifocal Leukoencephalopathy

C. Multiple sclerosis

D. Myasthenia gravis

64. Patient comes with bone pain, hypercalcemia, and renal failure. Bone marrow biopsy shows >10% (or extensive) clonal plasma cells. What is the most likely diagnosis?

A. Multiple myeloma

B. Waldenstrom macroglobulinemia

C. Acute myeloid leukemia (AML)

D. Primary hyperparathyroidism

65. Child presents with generalized edema, diagnosed as nephrotic syndrome. Renal biopsy findings show normal glomeruli on light microscopy, negative immunofluorescence, and electron microscopy shows podocyte effacement. Diagnosis:

A. Minimal change disease

B. FSGS

C. IgA nephropathy

D. Membranous glomerulonephritis

66. A 30-year-old patient with thyroid swelling. On biopsy, there are nuclei with cleared chromatin ("Orphan Annie eye") and psammoma bodies. What is the most likely diagnosis?

A. Follicular thyroid carcinoma

B. Papillary thyroid carcinoma

C. Medullary thyroid carcinoma

D. Anaplastic thyroid carcinoma

67. Patient presents with dyspnea and hemoptysis, and a lung mass/infiltrates on X-ray. On biopsy, there are non-caseating granulomas with Langhans giant cells. The most likely diagnosis is?

A. Tuberculosis

B. Lung cancer

C. Histoplasmosis

D. Sarcoidosis

68. A 3-year-old boy presents with a large, smooth left abdominal mass. CT scan is performed. Most likely diagnosis?

A. Neuroblastoma

B. Wilms tumor (Nephroblastoma)

C. Hepatoblastoma

D. Teratoma

69. Patient presents with progressive exertional dyspnea. CT shows subpleural honeycombing appearance. Diagnosis?

A. Usual interstitial pneumonia (UIP / IPF)

B. Desquamative interstitial pneumonia

C. Non-specific interstitial pneumonia

D. Cryptogenic organizing pneumonia

Microbiology

70. A 20-year-old female 2 weeks after delivery is lactating, comes complaining of a painful breast mass associated with high temperature. The causative organism is Gram-positive cocci in clusters, catalase positive, coagulase positive. What is the organism?

A. Staphylococcus aureus

B. Streptococcus pyogenes

C. E. coli

D. Staphylococcus epidermidis

71. A 30-year-old patient with a painless or painful vulvar ulcer and matted/enlarged inguinal LN (Lymphogranuloma venereum). The most likely causative organism is:

A. Treponema pallidum

B. Haemophilus ducreyi

C. Chlamydia trachomatis (serovars L1-L3)

D. Neisseria gonorrhoeae

72. A 29-year-old female presents with vulvar lesions (Condylomata acuminata). Most likely cause is which human papillomavirus (HPV) type?

A. Types 16, 18

B. Types 6, 11

C. Types 31, 33

D. Types 18, 45

73. Pregnant lady with varicella zoster infection. Regarding Congenital Varicella Syndrome, which gestational period carries the highest risk of this specific syndrome?

A. Only first 8 weeks

B. From 8 weeks till 20 weeks

C. Between 20 weeks and 24 weeks

D. Last trimester

74. Newborn with neonatal sepsis. Organism found is Gram-positive cocci, coagulase-negative, catalase-positive, and Novobiocin sensitive (commonly causing catheter infections). What is the organism?

A. Staphylococcus aureus

B. Staphylococcus epidermidis

C. Staphylococcus saprophyticus

D. Streptococcus agalactiae

75. Pregnant woman tests positive for Parvovirus B19 IgM and negative IgG, and the fetus subsequently develops hydrops fetalis. What is the underlying pathophysiology?

A. Viral destruction of fetal erythroid progenitor cells leading to severe aplastic anemia and high-output cardiac failure.

B. Immune-mediated hemolysis caused by maternal IgG cross-reacting with fetal red blood cell membranes.

C. Direct viral-induced damage to the fetal renal tubules causing severe proteinuria and fluid shifts.

D. Obliteration of the placental villous vasculature leading to severe pre-placental hypoxia.

76. Syphilis is a disease caused by Treponema pallidum. What is true about this organism?

A. VDRL test is highly specific for confirmation

B. Cannot be seen by standard Gram stain but visualized under dark-field microscopy

C. It is easily cultured on standard chocolate agar plates

D. It is a Gram-positive bacillus

77. Untreated neonatal HSV infection typically presents as:

A. Localized skin/eye/mouth disease with low mortality

B. Disseminated or CNS disease with extremely high mortality rates if untreated

C. Asymptomatic carriage in 90% of cases

D. Isolated cardiac abnormalities

78. What classic findings are caused by congenital Cytomegalovirus (CMV)?

A. Sensorineural deafness, microcephaly, and periventricular calcifications

B. Nasal hypoplasia and stippled epiphyses

C. Chorioretinitis, hydrocephalus, and diffuse intracranial calcifications

D. Cataracts, patent ductus arteriosus (PDA), and blueberry muffin rash

79. What is the most common viral cause of congenital anomalies worldwide?

A. Rubella

B. Herpes simplex virus

C. Cytomegalovirus (CMV)

D. Varicella Zoster virus

80. A 2-day-old neonate presents with early-onset neonatal sepsis. What is the most common causative organism transmitted via the birth canal?

A. Staphylococcus aureus

B. Streptococcus agalactiae (Group B Strep)

C. Streptococcus pneumoniae

D. Listeria monocytogenes

81. Listeria monocytogenes is a facultative intracellular bacterium characterized by "tumbling motility". What is its primary virulence factor used to escape phagosomes?

A. Streptolysin O

B. Pili

C. Listeriolysin O

D. Cord factor

82. Patient with fever and lymph node enlargement. Biopsy shows atypical lymphocytes (Downey cells) and Monospot test is positive. What cell type does this pathogen primarily infect and via which receptor?

A. T lymphocytes via CD4

B. B lymphocytes via CD21 (CR2)

C. NK cells via CD16

D. Dendritic cells via DC-SIGN

83. Patient with recurrent UTIs and staghorn kidney stones. The pathogen is Gram-negative, highly motile (swarming), and Urease positive. What is the organism?

A. E. coli

B. Proteus mirabilis

C. Klebsiella pneumoniae

D. Staphylococcus saprophyticus

84. A 12-week pregnant woman presents with a UTI due to E. coli. She has a severe allergy to penicillin. What is an appropriate alternative management?

A. Cephalexin (if non-anaphylactic) or Nitrofurantoin

B. Nitrofurantoin (safe in 1st/2nd trimester)

C. Amoxicillin-clavulanate

D. Trimethoprim (contraindicated in 1st trimester due to antifolate effect)

85. Which form of Chlamydia trachomatis is the infectious, metabolically inactive form?

A. Reticulate body

B. Elementary body

C. Dividing form

D. Collateral body

Clinical

86. A pregnant lady at 18 weeks suddenly develops severe dyspnea during a procedure/ admission. Her pO2 is 84%, ECG shows right heart strain, and D-dimer is positive. What is the most likely cause?

A. Pulmonary edema due to fluid overload

B. Congestive heart failure

C. Acute Pulmonary Embolism (PE)

D. Amniotic fluid embolism

87. A gravida at 39 weeks presents in active labor with a history of ruptured membranes. The cervix remains at 5-6 cm dilated for 2 hours despite adequate oxytocin augmentation. What is the next step in management?

A. Artificial rupture of membranes (if not fully ruptured)

B. Immediate Cesarean Section due to arrest of dilation

C. Continue oxytocin and re-assess in 4 hours

D. Double the oxytocin dose rapidly

88. An asymptomatic pregnant woman at 20 weeks is found to have a short cervical length of 18 mm on ultrasound. What is the most appropriate initial management?

A. Oral progesterone

B. Emergency cervical cerclage

C. Vaginal progesterone supplementation

D. Absolute bed rest and hospitalization

89. Patient with massive postpartum hemorrhage (PPH) losing 2 liters of blood within 1 hour after a Cesarean section. The uterus is atonic and not responding to medical uterotonics. What surgical step is typically performed first to compress the uterus before proceeding to hysterectomy?

A. B-Lynch uterine compression suture

B. Uterine artery ligation

C. Internal iliac artery ligation

D. Intrauterine balloon tamponade (if not already tried)

90. Patient with severe uterine bleeding 30 minutes after vaginal delivery. On exploration, there is no retained placenta and no lower genital tract lacerations, and the uterine fundus is found to be firm and well-contracted. What is the most likely cause?

A. Disseminated intravascular coagulation (DIC) / Coagulopathy

B. Uterine atony

C. High cervical laceration not visible on routine speculum exam

D. Retained placental fragments

91. A 45-year-old female with heavy menorrhagia is diagnosed with a submucosal fibroid. She strongly wishes to avoid surgery. What medical line of treatment is suitable to control the bleeding?

A. Oral Mefenamic acid and Tranexamic Acid during the cycle

B. Levonorgestrel-releasing intrauterine system (Mirena)

C. Combined oral contraceptive pills

D. GnRH agonists short-term

92. Female complains of severe lower abdominal pain, pelvic tenderness, and a temperature of 38.6°C. Clinical findings confirm Pelvic Inflammatory Disease (PID). What is the appropriate outpatient or inpatient antibiotic regimen?

A. Oral Doxycycline + Metronidazole for 14 days

B. Single dose of IM Ceftriaxone + oral Doxycycline + Metronidazole for 14 days

C. Single dose of IM Ceftriaxone + single dose of Azithromycin

D. Inpatient IV Cefoxitin + IV Doxycycline

93. A pregnant woman is a known case of Antiphospholipid Syndrome (APS). What is the recommended thromboprophylaxis regimen during pregnancy?

A. Aspirin 75–150 mg daily alone

B. Low-dose Aspirin + Low Molecular Weight Heparin (LMWH) throughout pregnancy

C. High-dose Low Molecular Weight Heparin alone

D. Warfarin throughout pregnancy, switching to LMWH near delivery

94. A laboring patient suddenly develops umbilical cord prolapse. On vaginal examination, the cord is felt protruding below the presenting part. What is your immediate action?

A. Put the mother in Trendelenburg position only and wait for help

B. Manually elevate the presenting part, fill the bladder (Vago's method) to relieve pressure, and prepare for emergency C-section

C. Gently push the prolapsed cord back into the uterine cavity and manage expectantly

D. Perform immediate operative vaginal delivery (forceps) regardless of cervical dilation

95. A 28-year-old lady presents with secondary amenorrhea. Her labs show: FSH 28 IU/L, LH 18 IU/L, Estradiol 0.8 pg/mL. What is the most likely diagnosis?

A. Hypothalamic amenorrhea

B. Hypergonadotropic hypogonadism (Premature Ovarian Insufficiency / Failure)

C. Polycystic Ovary Syndrome (PCOS)

D. Early pregnancy

96. A pregnant lady presents at 36 weeks with decreased fetal movements. CTG shows a distinct sinusoidal pattern. What critical underlying condition must be excluded/managed immediately?

A. Normal fetal sleep cycle

B. Maternal opioid or sedative intake

C. Chronic uteroplacental insufficiency

D. Severe fetal anemia (e.g., due to Rh isoimmunization or fetomaternal hemorrhage)

97. A pregnant lady at 24 weeks gestation presents with preterm uterine contractions. Which tocolytic agent is most appropriate and commonly preferred in this trimester?

A. Oral Nifedipine

B. IV Magnesium sulfate (primarily for neuroprotection <32 weeks)

C. IV Atosiban (Oxytocin receptor antagonist) or Nifedipine

D. Ritodrine (Beta-2 agonist)

98. A 24-year-old female at 8 weeks from LMP has a positive pregnancy test. Transvaginal US shows an empty uterus and a 4 cm adnexal mass. Serum beta-hCG is 1800 mIU/mL. The patient is hemodynamically stable. Based on the concept of the Discriminatory Zone, what is the correct interpretation/action?

A. Repeat serum hCG after 48 hours to assess the trend

B. hCG is below the discriminatory zone, so Methotrexate is strictly contraindicated

C. Since beta-hCG is above the discriminatory zone (typically 1500) and no IUP is seen, it confirms a highly probable ectopic pregnancy

D. Treat empirically with surgical laparoscopy immediately

99. A preeclamptic pregnant woman at 28 weeks on Magnesium Sulfate infusion suddenly develops loss of deep tendon reflexes (areflexia) and bradypnea (RR 9 breaths/min). What is the immediate life-saving action?

A. Stop Magnesium Sulfate immediately, administer oxygen, and monitor fluids

B. Stop infusion and administer IV Calcium Gluconate 10% (10 mL over 5-10 minutes)

C. Administer IV Calcium Phosphate immediately

D. Intubate the patient immediately and prepare for emergency Cesarean section

100. A pregnant lady presents at 41 weeks + 3 days with no signs of labor. Her Bishop score is evaluated as 3 (unfavorable cervix). What is the most appropriate initial method for induction of labor?

A. Oxytocin infusion

B. Artificial rupture of membranes (Amniotomy)

C. Prostaglandin E2 (Dinoprostone gel/insert)

D. High-dose oral Misoprostol

101. During a vaginal delivery, shoulder dystocia occurs. After failure of the McRoberts maneuver and application of suprapubic pressure, what is typically the next appropriate internal manipulation maneuver?

A. Zavanelli maneuver (cephalic replacement)

B. Symphysiotomy

C. Delivery of the posterior arm or Wood's screw maneuver

D. Intentional cleidotomy (fracture of the clavicle)

102. A patient at 34 weeks gestation presents with a BP of 160/100 mmHg, blurred vision, severe headache, epigastric pain, platelets 76,000/microL, and significantly elevated AST. What is your immediate management plan?

A. Administer MgSO4 for seizure prophylaxis, control BP with antihypertensives, give corticosteroids, and plan for prompt delivery (HELLP syndrome)

B. Proceed to immediate emergency Cesarean section without stabilization

C. Give IV Hydralazine alone and observe for 48 hours

D. Outpatient management with close home blood pressure monitoring

103. In the third stage of labor, the placenta has not separated after 20 minutes of active management. What is the next appropriate step?

A. Immediate manual removal of placenta under general anesthesia

B. Ensure bladder is empty, administer an additional dose of oxytocin, and continue controlled cord traction gently

C. Injection of oxytocin directly into the umbilical vein

D. Wait expectantly for a total of 60 minutes before any intervention

104. A postpartum patient develops sudden severe shortness of breath, pleuritic chest pain, and hemoptysis. D-dimer is elevated. What is the gold-standard diagnostic investigation of choice?

A. Ventilation-perfusion (V/Q) scan

B. CT Pulmonary Angiography (CTPA)

C. Portable Chest X-ray

D. High-resolution CT chest

105. A 26-year-old woman at 36 weeks gestation screens positive for Group B Streptococcus (GBS) via vagino-rectal culture. She has a severe penicillin allergy with a history of documented anaphylaxis. Susceptibility testing is unavailable. What is the preferred intrapartum antibiotic prophylaxis?

A. Ampicillin

B. Erythromycin

C. Clindamycin (only if susceptible) or Vancomycin

D. Cefazolin (contraindicated if history of true anaphylaxis to penicillin)

106. A twin pregnancy at 26 weeks is diagnosed with Twin-to-Twin Transfusion Syndrome (TTTS) Stage II/III, confirmed by polyhydramnios in the recipient and oligohydramnios in the donor. What is the definitive first-line treatment of choice?

A. Serial amnioreduction alone and delivery at 28 weeks

B. Immediate administration of corticosteroids and urgent Cesarean section

C. Fetoscopic laser ablation of the communicating placental vascular anastomoses

D. Expectant management with weekly Doppler ultrasounds

107. A multiparous woman in active labor at 7 cm dilation. CTG reveals a prolonged deceleration down to 70 bpm lasting for 4 minutes. Fetal head is at station +1. What is the immediate initial management?

A. Prepare for immediate emergency Cesarean section

B. Turn patient to left lateral position, stop oxytocin, initiate IV fluid bolus, and administer oxygen (intrauterine resuscitation)

C. Attempt immediate instrumental vaginal delivery (forceps/vacuum)

D. Perform fetal scalp pH sampling

108. A pregnant woman is a known case of HIV on Antiretroviral Therapy (ART). Her CD4 count is 450 cells/microL and her viral load is undetectable near delivery. What is the recommended delivery plan?

A. Continue ART, plan for Normal Vaginal Delivery (NVD) if obstetric criteria are met, and provide neonatal prophylaxis

B. Stop current ART, change to Zidovudine-based monotherapy, and plan C-section at 37 weeks

C. Scheduled Cesarean section at 38 weeks and mandatory intrapartum IV Zidovudine

D. Add Zidovudine intraoperatively to the baby only

109. A 45-year-old female presents with heavy menorrhagia. Transvaginal ultrasound reveals an endometrial thickness of 12 mm. What is the next most appropriate diagnostic step?

A. Hysteroscopy with directed endometrial biopsy

B. Outpatient endometrial biopsy (e.g., Pipelle biopsy)

C. Total abdominal hysterectomy

D. Reassurance and repeat ultrasound in 3 months

Pharmacology

110. A female patient being treated for secondary syphilis develops a Jarisch-Herxheimer reaction within hours of receiving her first dose of intramuscular Penicillin G. This reaction is characterized by:

A. An acute IgE-mediated allergic hypersensitivity to penicillin

B. Acute onset of fever, chills, headache, myalgia, tachycardia, and exacerbation of skin lesions due to endotoxin-like release from dying spirochetes

C. Rapid development of bacterial resistance to penicillin

D. Drug-induced acute autoimmune hemolytic anemia

111. Methotrexate is a chemotherapeutic/antimetabolite agent that acts by competitively inhibiting the dihydrofolate reductase (DHFR) enzyme. As a result, which active substance is directly prevented from forming?

A. Tetrahydrofolate (THF)

B. Dihydrofolate (DHF)

C. Folic acid

D. Adenosine triphosphate

Statistics

112. A clinical trial reports that the Number Needed to Treat (NNT) for a new antihypertensive drug is 20 to prevent one cardiovascular event. This means:

A. Twenty patients must be treated with the drug for a specific duration to prevent one adverse cardiovascular event

B. Twenty patients will develop side effects for every one person who benefits

C. Out of 100 treated patients, exactly 20 will achieve normal blood pressure

D. The drug requires 20 weeks of continuous treatment before any clinical benefit is observed

113. A new screening test has a Sensitivity of 90% and a Specificity of 80%. The prevalence of the disease in the population is 1%. What is the approximate Positive Predictive Value (PPV) of this test?

A. 80%

B. 4.3% (approx. 4–5%)

C. 90%

D. 1%

114. In a clinical study comparing an active drug against a placebo, the primary outcome yields a p-value = 0.03. This means:

A. There is a 3% probability that the null hypothesis is true

B. There is a 97% probability that the drug is highly effective in the general population

C. The study results are guaranteed to be highly clinically significant

D. There is a 3% probability of obtaining the observed results (or more extreme) by chance alone, assuming the null hypothesis is true

115. A screening test possesses a high Sensitivity (90%) but low Specificity (under 60%). For which clinical purpose is this test most useful?

A. Confirming a definitive diagnosis in a high-risk patient

B. Screening a population to effectively rule out the disease in those who test negative

C. Limiting the number of false-positive results

D. Serving as a gold-standard confirmatory test

116. In a case-control study evaluating smoking and lung disease, the Odds Ratio (OR) is calculated to be 8.0. What is the correct interpretation of this finding?

A. The odds of having a history of smoking are 8 times higher among cases (diseased) than among controls (non-diseased)

B. Exactly 8% of all smokers will eventually develop the lung disease

C. All smokers will develop the disease except for an 8% subset

D. Smoking decreases the risk of developing lung disease by a factor of 8

117. The mid-cycle FSH surge (occurring alongside the dominant LH surge) is physiologically important because it:

A. Triggers final oocyte maturation entirely independently of LH

B. Upregulates LH receptors on granulosa cells and stimulates plasminogen activator production required for follicular rupture

C. Stimulates estrogen production to continue the positive feedback loop

D. Induces immediate regression of the corpus luteum

اللهمَّ يا معلِّمَ آدمَ، ويا فهَّامَ سليمانَ، ويا مؤتِيَ لُقمانَ الحكمة وفصل الخطاب، علّمنا ما ينفعنا، وانفعنا بما علمتنا، وزدنا علمًا، وافتح لنا أبواب الفهم، ويسِّر لنا سبل التفوق والنجاح.